

Weight management program bullseye research report

📄 Tzuyi Lee and [1 more](#) Published 5 May 2026 This doc was generated using AI

...

Weight Management Bullseye Research

Created with FigJam

Figma

4 participants · 3 concepts tested · Jennifer Jones, Ron Riemer, Linda Campbell, Robin English

How to read this report

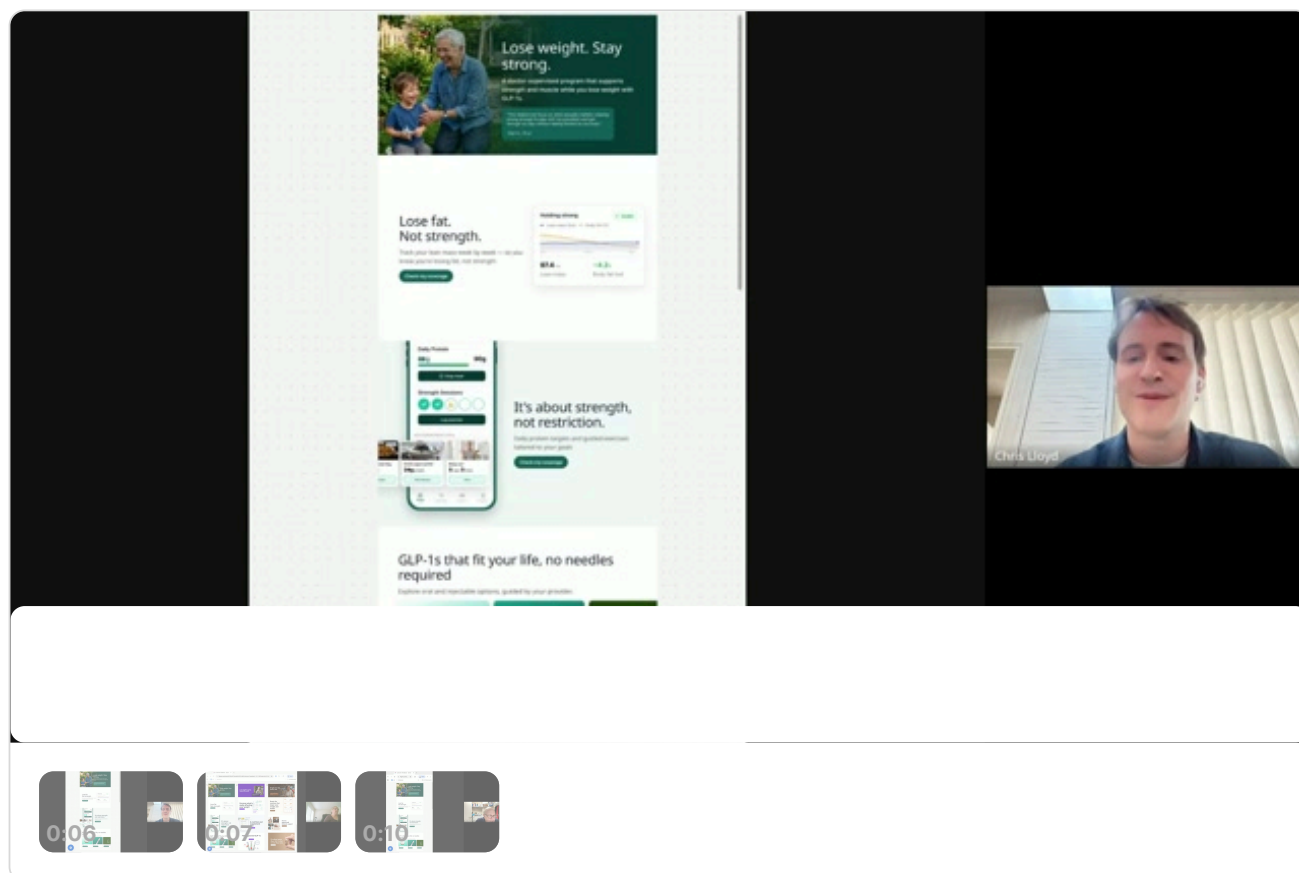
Insights are ordered by **signal strength** — the number of participants who mentioned or reacted to the theme, and the strength of their sentiment. Each insight includes direct quotes and the source session. The final section covers **what to deprioritize**.

 **STRONG SIGNAL — Clear direction, act on this**

1. "Strength, Not Restriction" — The single most resonant message across the board

4/4 participants reacted positively to this framing

This headline from Concept 1 was the most broadly resonant phrase across all sessions — regardless of which concept participants ultimately preferred. It speaks to a shared fear (being told what you can't eat) and reframes the goal around gain, not deprivation.



"Like what it says, it says about strength, not restriction. That's what I'm talking about."

— Jennifer Jones

"It's about strength, not restriction... there's a lot of foods that I love, and if I have a restriction..."

— Ron Riemer

"Oh, the Strength, Not Restriction here was kind of interesting."

— Linda Campbell

"It's not just losing weight, but it is including the features that I talked about about staying strong, about supporting your strength and muscle."

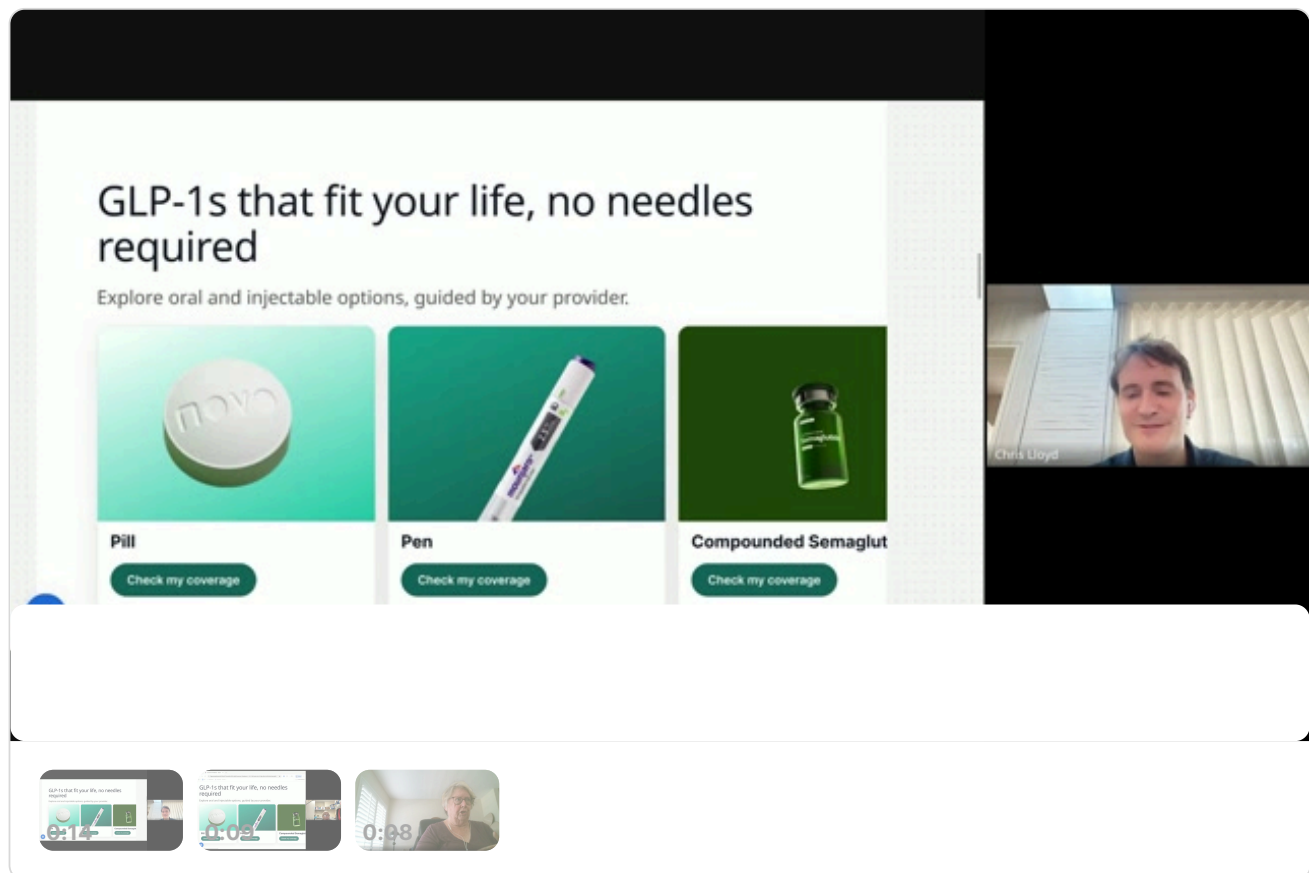
— Robin English

Next step: This framing should anchor the core value proposition across all concepts. It should not be isolated to Concept 1. Test whether it can be carried into the clinical (Concept 2) and lifestyle-tracking (Concept 3) framing as well.

2. Pill format is a prerequisite — injections are a hard blocker for most

4/4 participants raised this unprompted or with strong feeling

Every participant expressed a clear preference for a pill form of GLP-1, with vial-and-needle injection being a firm rejection. This is not just a preference — for multiple participants, seeing an injectable option was the thing that killed interest in an otherwise compelling concept.



"I'll do it. I mean, I'll do a pill. Like I said, I don't have to think about it. And I take, I take pills anyway."

— Jennifer Jones

"Definitely pill one, pen two... Compounded with a vial like that, no way."

— Ron Riemer

"At first it was the shot. Now it's the pill. I mean, much easier to do the pill for me because I'm deathly afraid of shots."

— Linda Campbell

"I would rather it be the approved FD, you know, the Lily Direct or the, you know, whoever is actually done the research on it because they haven't done the research on all these compounded semiglut types."

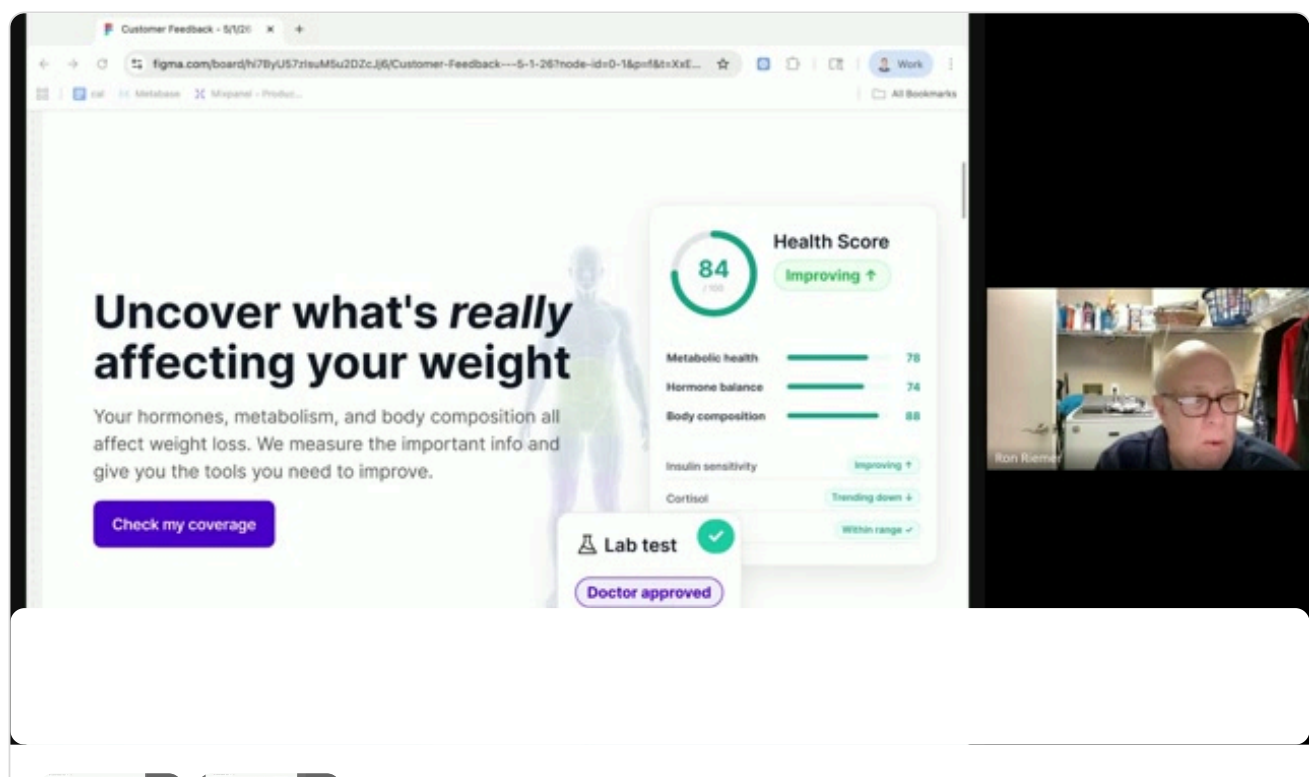
— Robin English

Next step: The pill-first GLP-1 option needs to be front and center in any concept. Compounded injectables should either be removed or placed far down the page, as they are actively undermining trust and conversion for multiple participants.

3. Insurance coverage + "Check My Coverage" — a trust anchor and conversion trigger

4/4 participants reacted to this; 2 said it could drive plan-switching

The visibility of an insurance check across all concepts was noted as critical. For participants on Medicare Advantage, knowing coverage was possible transformed their reaction from skeptical to enthusiastic. One participant said he would switch insurance plans to access this program.





"It was approved by the FDA and my insurance probably cover it. I'm like, woo-hoo."

— Jennifer Jones

"Now also, all, all this one and the one before, it says check my coverage. That's important."

— Ron Riemer

"It's been that they were so expensive that you know, it wasn't something I even wanted to think about, but now I guess they're coming down."

— Linda Campbell

"Cost is a big concern but also you know the number of side effects and just that we don't know like what is it 20% of the US population's on it or something now."

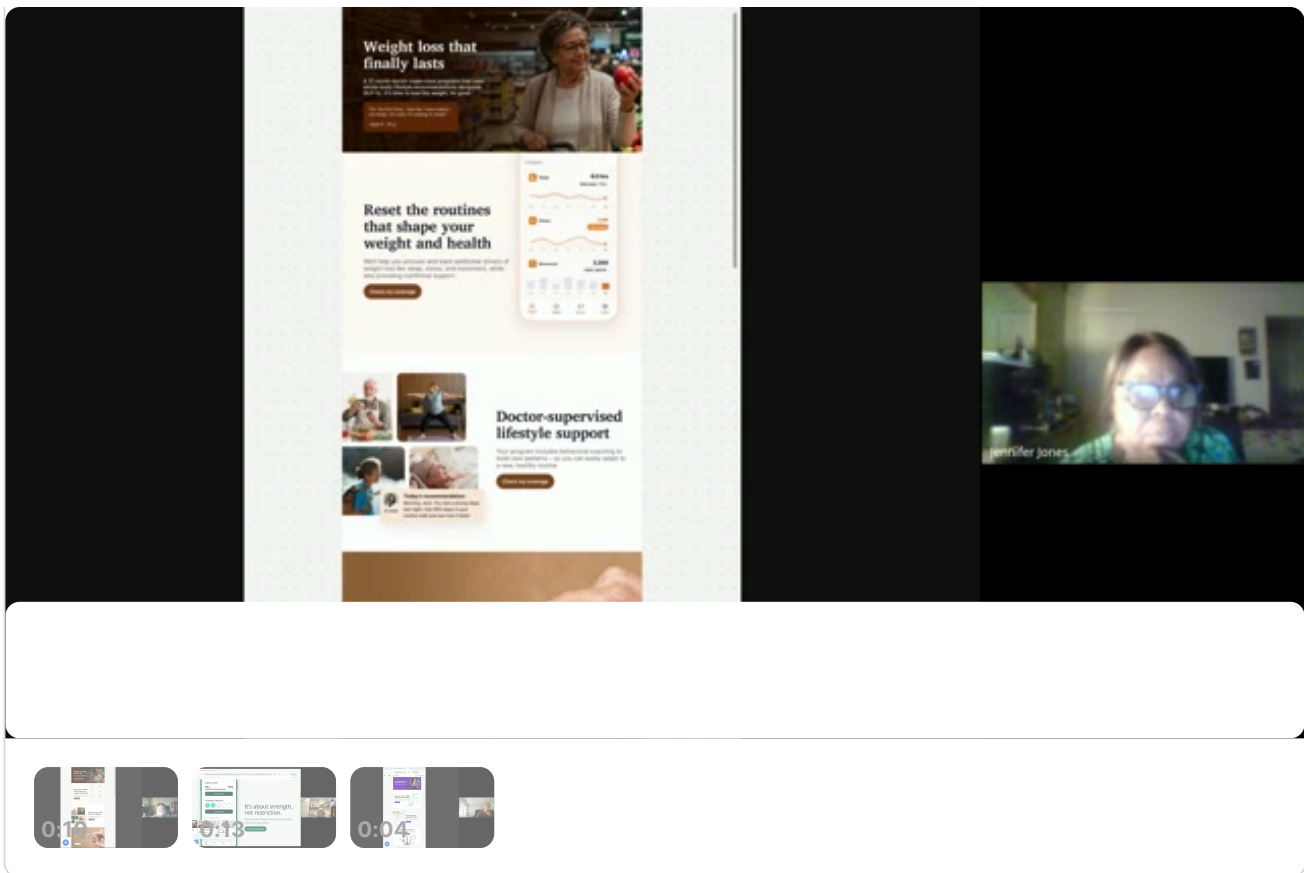
— Robin English

Next step: "Check My Coverage" should appear early and prominently in all concepts — not buried at the bottom. Given Medicare coverage changes in July, this could be a time-sensitive acquisition wedge. Explore making this interactive upfront rather than a passive CTA.

4. Doctor/human supervision = motivation, not just safety

3/4 participants called this out as a primary motivator (Jennifer, Ron, Linda)

Participants don't just want supervision for clinical oversight — they want it because it keeps them going. The framing of "not being alone in this" was emotionally resonant. A coach or nutritionist was preferred over a physician-only model by some.



"Because I'm not in it by myself... without that doctor supervise, I'm like, uh, you know, I don't feel so well, maybe I should just quit. Whereas the doctor can be alerted to that and do something maybe different."

— Jennifer Jones

"If I can get on a program with some help from, to be able to talk to some type of person online like you, to work me through it, maybe we could handle that."

— Ron Riemer

"Yeah, just to get me started and then periodically [having a real person offering guidance]."

— Linda Campbell

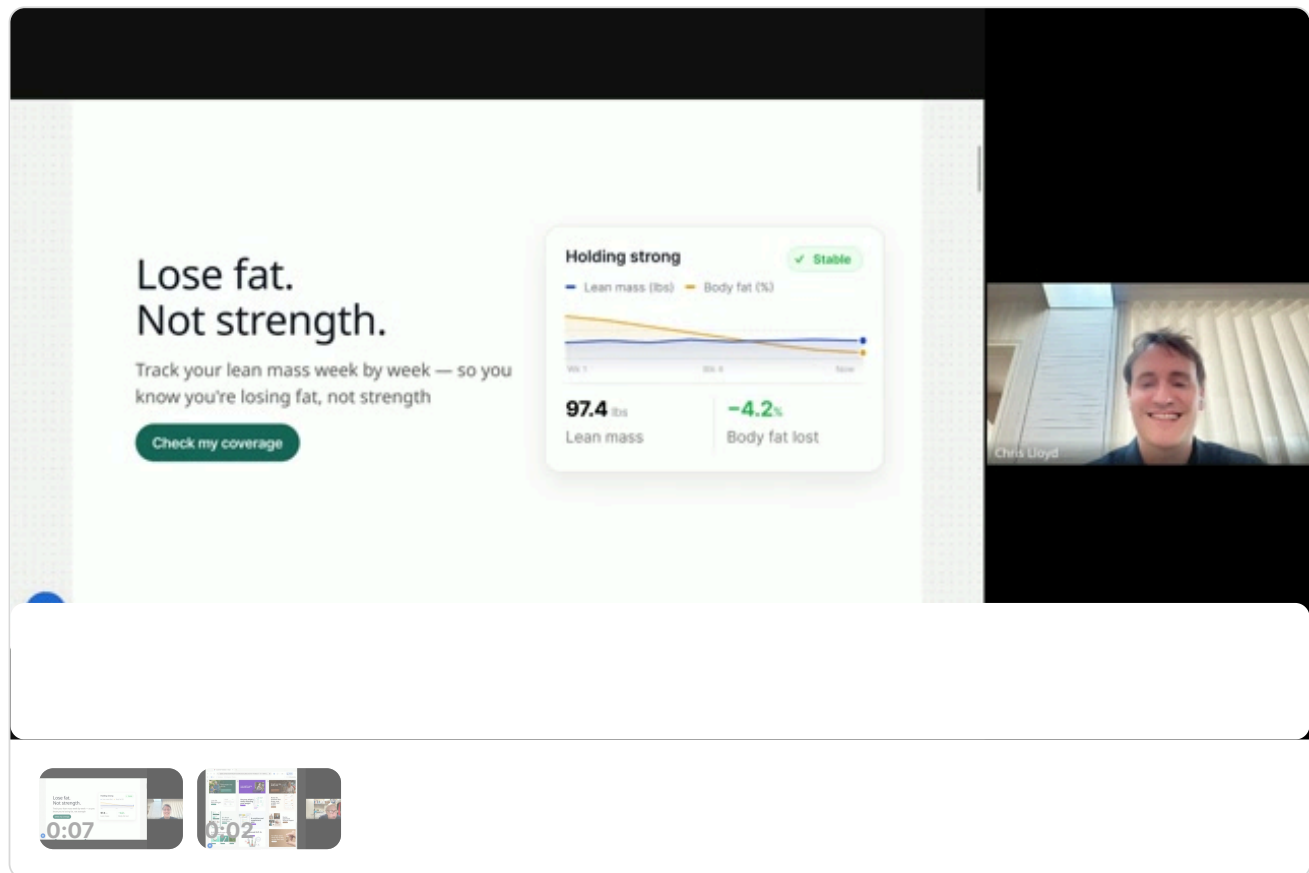
Next step: Supervision messaging needs to go beyond "doctor-supervised" as a credential and communicate the motivational benefit — someone who notices, adjusts, and keeps you going. Explore whether a nutritionist or coach role (not just a physician) is a meaningful differentiator.

🟡 MODERATE SIGNAL — Directional, warrants deeper exploration

5. Lean mass / body composition tracking — novel and differentiated, but "how?" is a barrier

3/4 participants found this interesting (Jennifer, Robin, Ron)

Tracking fat loss vs. muscle loss was called out as something participants had never seen in another program. This is a potential differentiator — especially for participants worried about GLP-1 side effects on muscle and bone. However, all three had an immediate follow-up: "How does that actually work?"



"Track you? Yeah. I mean, how do you track your lean mass? How do you do that?"
— Jennifer Jones

"I've not seen that kind of a feature on other programs."
— Robin English

"I like the part that's monitoring your fat loss versus your muscle mass because I haven't seen a program that has that kind of information available."

— Robin English

"Lose fat but not strength. That's good. Most of my weight is from my stomach up to about here."

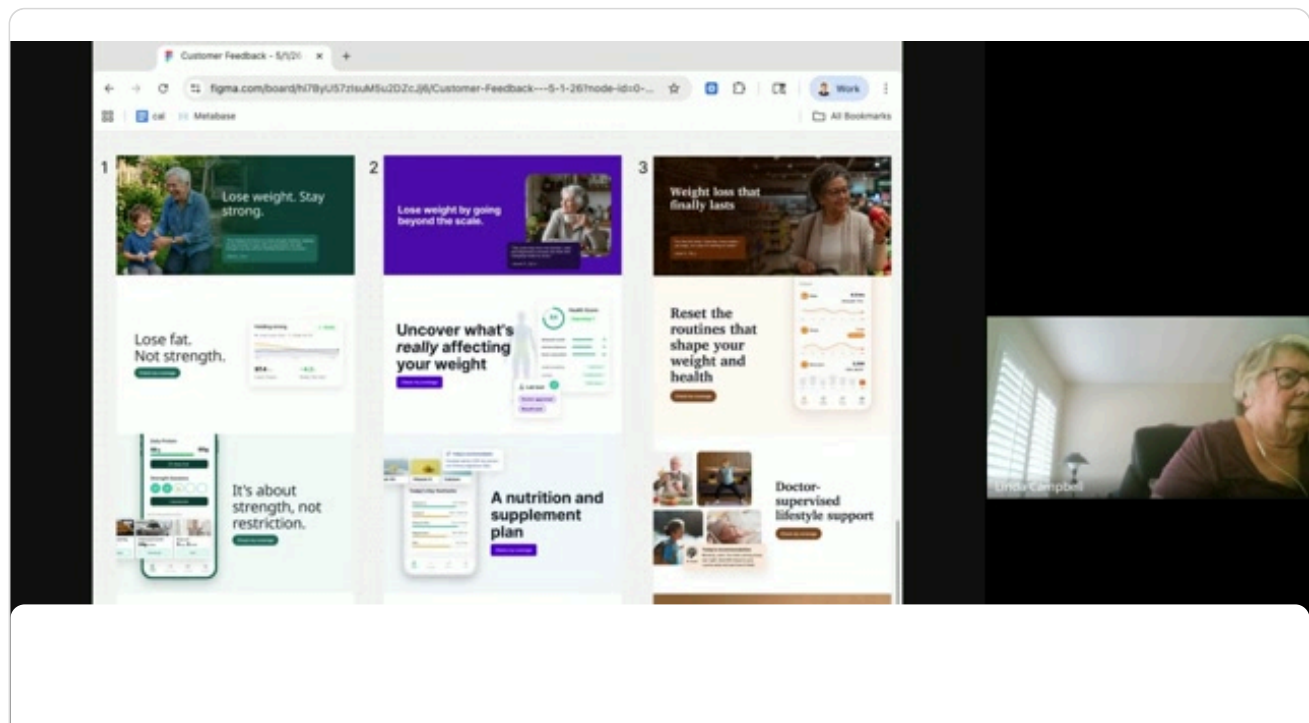
— Ron Riemer

Next step: The feature is compelling but the prototype does not adequately explain how it works. Test clearer explanations of the measurement method (e.g., phone scan, DEXA, wearable) to see if removing the "black box" concern increases confidence and conversion intent.

6. Lab-based clinical depth (Concept 2) — the most "premium" feel, but needs a stronger headline

2/4 participants preferred it (Linda, Robin); Robin flagged the headline as weak

Metabolic health, hormone balance, insulin sensitivity, and cortisol markers were seen as going beyond what a primary care doctor typically offers. Participants who valued comprehensiveness were most drawn to this concept. However, Robin called the headline "Lose weight by going beyond the scale" generic and weak for what the concept actually offers.





"I've not seen any kind of measurement of that in any weight loss program really, but those are good markers of your general health too."

— Robin English

"Theoretically, that sounds more exact, more precise."

— Robin English

"I guess probably this middle one, number 2, because of the fact it's got the nutrition and supplement plan shown."

— Linda Campbell

"Lose weight by going beyond the scale. I think that needs a stronger headline there because that's so generic."

— Robin English

Next step: The content of Concept 2 is strong, but the headline undersells it. Explore headlines that communicate the clinical depth more directly. Also test whether combining the lean mass tracking from Concept 1 with the lab-based approach from Concept 2 creates a clearly superior offer — multiple participants asked for exactly this combination.

7. Microdosing / taper approach — only one participant responded enthusiastically, but it filled a real gap

1/4 with strong enthusiasm (Robin); positive but softer reactions from Jennifer

Microdosing as a lower-risk entry into GLP-1 use generated Robin's most unprompted positive response in the session. The logic — fewer side effects, easier tapering, less dependency — resonated with the general anxiety about long-term GLP-1 use across participants. Its absence from Concept 2 was flagged as a missed opportunity.

It's about more than
GLP-1s but we do
that, too.

Our program starts with lifestyle changes. If you need a weight loss boost, we'll create a plan for GLP-1s that start with the lowest effective dose and a plan to taper off as your new habits take over.

[Check my coverage](#)



"You'll have probably less problems with side effects and tapering off of it'll probably be easier because you're not getting your body used to so much."
— Robin English

"I don't know why they couldn't offer the microdosing along with it."
— Robin English

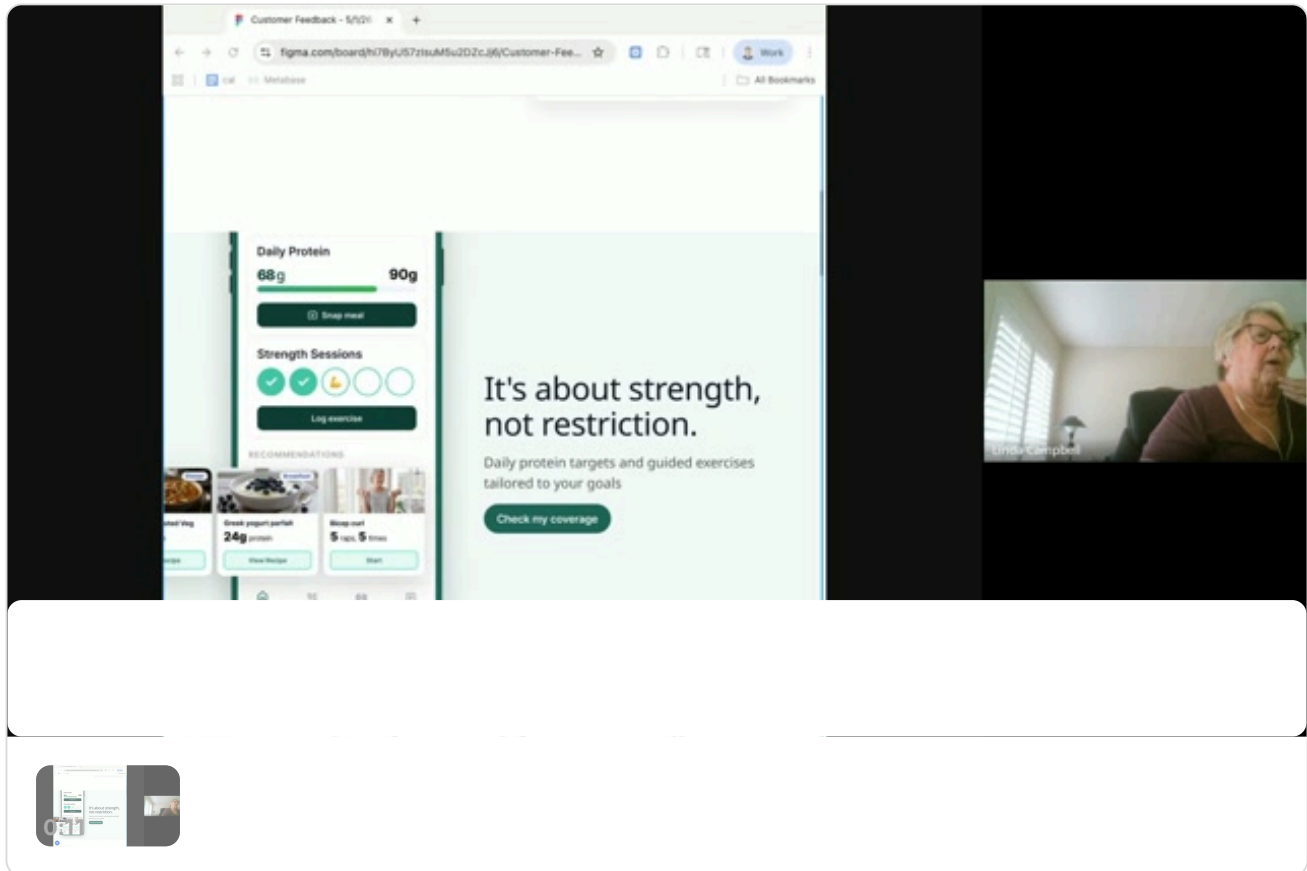
"I think that's good. It's kind of like, see how this works for me. If I get any sickness or big side effects, and then if not, could increase the dosage over time."
— Jennifer Jones

Next step: The taper/microdose approach addresses a real emotional concern (indefinite dependency on a drug). Consider whether this framing should be present across *all* concepts as a GLP-1 entry philosophy, not just Concept 3. This could help reduce GLP-1 hesitancy earlier in the decision journey.

8. Structured meal guidance wanted — but not food logging

3/4 participants (Linda, Robin, Jennifer) expressed some version of this

Participants want to be told what to eat ahead of meals — a plan, not a diary. The mental load of food logging was described as the primary reason existing tracking tools get abandoned. Snap-a-meal or proactive meal plan features are more compelling than retroactive logging.



"In advance of the meal is more helpful to have guidance rather than after the fact."
— Linda Campbell

"I'm exhausted. I don't want to cook today. I don't want to clean the kitchen. Here's what I have available. What can I make that would be okay that won't be a lot of effort?"
— Robin English

"I wouldn't want to have to put everything that I eat in there every day... that would be kind of very time-consuming."
— Linda Campbell

Next step: This is a feature design question more than a concept question. Explore whether a proactive "what should I eat today" guidance feature (vs. a food log) would

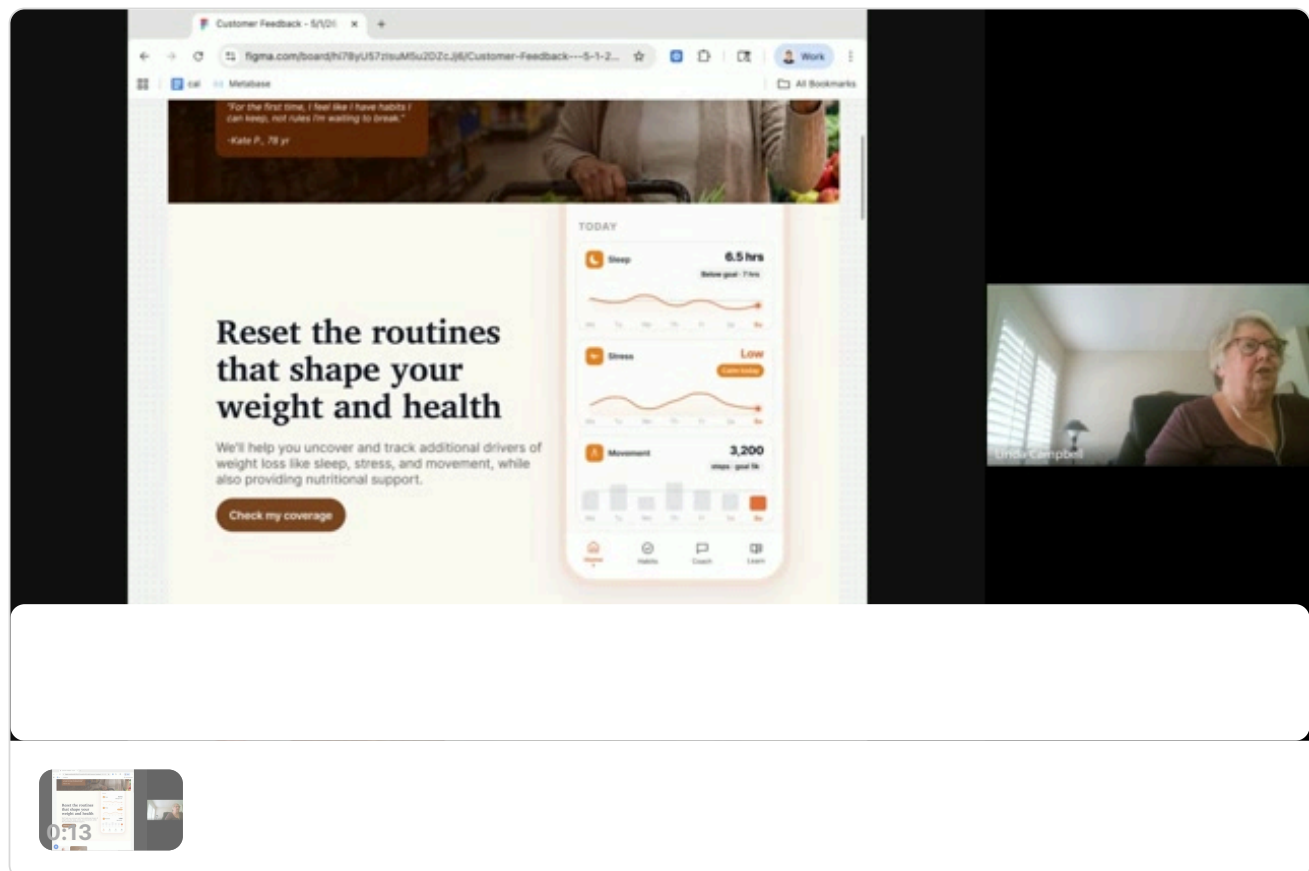
increase perceived value. Snap-a-meal was positively received by Robin — worth testing more explicitly.

● WEAK SIGNAL / CLEAR DIRECTION TO EXCLUDE

9. Generic lifestyle tracking (sleep, steps, activity) — perceived as widely available, low differentiation

2/4 participants actively dismissed this (Jennifer, Robin); Linda preferred not to wear a device

Sleep tracking, step counting, and general activity monitoring were seen as table stakes — something participants already have or can easily get for free. Wearable devices were unpopular. This was the least compelling part of Concept 3 for most participants.



"That's just a tracker, sleep and all that, and your steps and all that stuff. That's available."

— Jennifer Jones

"I don't know that I would pay for a program that this is the main thing they're offering when I know I can get that elsewhere."

— Robin English

"I have an Apple Watch and I never wear it, you know... I'm not into devices, the wearable devices at all."

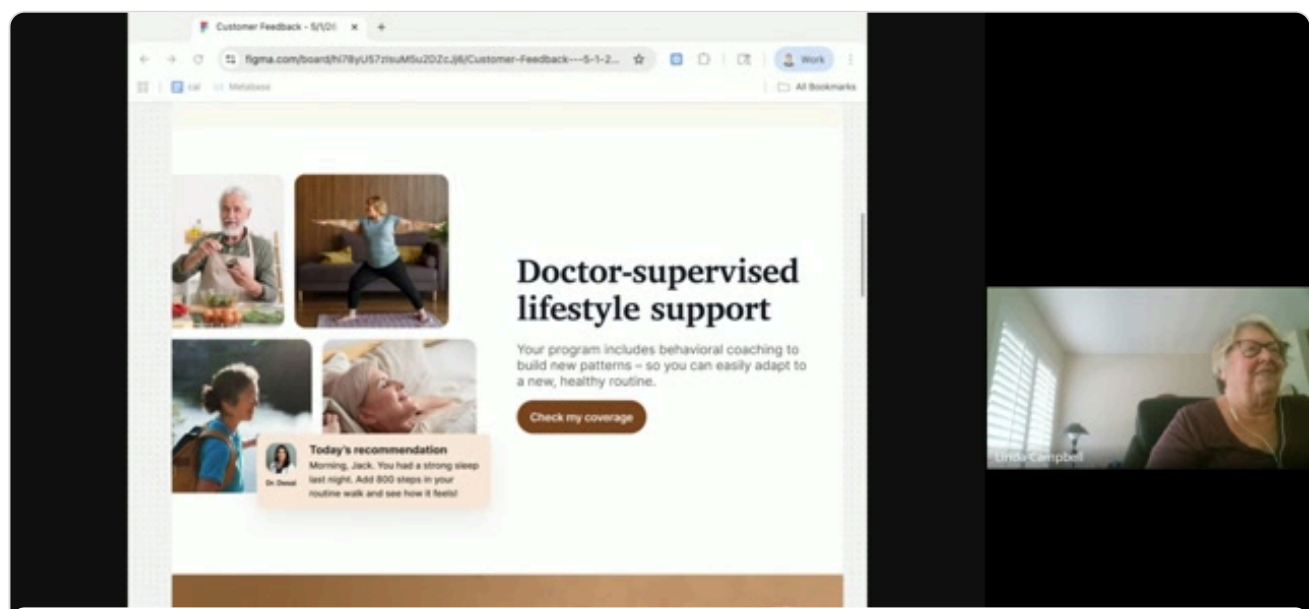
— Linda Campbell

Next step: Remove generic lifestyle tracking as a headline feature. It is actively weakening the perceived value of Concept 3. If included, it should be presented as a passive, background feature — not a selling point.

10. Physician chat check-ins (as standalone value) — not compelling without motivational framing

2/4 participants found it lower-value in isolation (Linda, Jennifer)

A physician chat was seen positively only when framed as motivational accountability (Jennifer). When framed as a routine check-in or lifestyle conversation, it felt unnecessary. Linda specifically said she doesn't want to chat all the time with people — she wants results.





"Probably not [useful]. I just want to see results and know that I'm doing it correctly, but I don't need to chat all the time with people."

— Linda Campbell

Next step: Don't lead with "physician chat" as a feature. Lead with the motivational outcome — "someone who keeps you on track" — and let the chat be the mechanism, not the headline.

Concept Preference Summary

Concept	Votes	Key Strengths	Key Weaknesses
Concept 1 — Lose Weight, Stay Strong	Jennifer ✓, Ron ✓, Robin (partial)	"Strength not restriction" messaging, lean mass tracking, doctor supervision	Compounded injectable, doesn't include FDA-only GLP-1 option
Concept 2 — Beyond the Scale (Lab-Based)	Linda ✓, Robin ✓	Lab-based clinical depth, personalized supplements, FDA-only GLP-1s	Weak headline, no lean mass tracking, no microdosing
Concept 3 — 12-Month Doctor Supervised	None voted first	Doctor supervision credibility	Lifestyle tracking feels generic, 12-month timeframe too slow for impatient users, injectable prominently featured

Composite ideal program (what participants designed for themselves): Concept 1's lean mass tracking + "strength not restriction" messaging + Concept 2's lab-based clinical depth + FDA-only GLP-1 options + microdose/taper philosophy + a human coach or nutritionist for accountability.

More insights in this project

[View insights](#)

Weight Management Program
Bullseye Research